

TO: The Registrar / Commissioner, Council for Medical Schemes

E: complaints@medicalschemes.co.za

REF: CMS-REQUEST-3 | High Court Case No. HCGS951842 | B-BBEE Commission Case No. 5/02/2026

DATE: 2 April 2026

RE: FORMAL FOLLOW-UP — JURISDICTIONAL RE-ENGAGEMENT REQUIRED — SYSTEMIC GEXUS-DMS INTEGRATION FAILURE CONSTITUTES MALADMINISTRATION UNDER THE MEDICAL SCHEMES ACT 131 OF 1998

Dear Registrar / Commissioner,

I write in formal follow-up to my correspondence of 20 March 2026 (CMS-REQUEST-3), which updated your office regarding the unremediated Gexus-DMS system integration failure at Medscheme Holdings (Pty) Ltd and its nexus to my urgent Section 162 Director Delinquency Application (HCGS951842, Western Cape High Court).

This letter serves to place the CMS on formal notice that the jurisdictional declination issued on 4 February 2026 — directing this matter to the Health Professions Council of South Africa — was premised on a characterisation of the disclosure as a complaint against a health professional. That characterisation does not reflect the substance of the matter now before this office. The disclosure concerns a systemic technical integration failure in Administrator infrastructure. As set out below, that failure falls squarely and inescapably within the CMS's statutory mandate.

I respectfully submit that a failure to engage substantively with this correspondence, in the context of a documented Priority 1 integration gap that continues to affect GEMS beneficiaries, would itself constitute a dereliction of the Council's statutory duty under the Medical Schemes Act 131 of 1998 ("the MSA").

1. REFRAMING THE DISCLOSURE: FROM PROFESSIONAL CONDUCT TO ADMINISTRATOR MALADMINISTRATION

The CMS's February 2026 declination was appropriate insofar as it related to any complaint against a health professional. That referral to HPCSA and SANC stands without objection.

However, the primary subject of this file is categorically distinct. The Gexus-DMS integration failure is a defect in Administrator infrastructure — the technical systems and processes through which Medscheme discharges its obligations as an Accredited Administrator under the MSA. Specifically:

- The Disease Management System (DMS) continued to flag GEMS member 000655789 Dependent 02 as non-adherent from December 2024 onwards, despite the member transitioning to the chronic benefit from January 2025 onward.
- The Gexus claims system recorded chronic benefit claims from April 2023 onward. DMS did not receive or recognise this update.
- As a result, automated non-adherence letters were dispatched on 19 May 2025 and 19 October 2025 — clinically incorrect communications sent to a compliant, elderly beneficiary.
- A Root Cause Analysis (RCA) confirmed the integration gap as a structural design flaw — not an isolated clerical error — affecting all members undergoing the acute-to-chronic benefit transition.

This is not professional misconduct. It is systemic Administrator maladministration within the precise meaning of Section 1 of the MSA, and it falls unambiguously within CMS jurisdiction.

2. THE CMS'S STATUTORY DUTIES — ENGAGED AND UNMET

Four distinct statutory obligations are triggered by this disclosure. Each is set out below.

2.1 Section 7 — Protection of Beneficiary Interests

The primary object of the CMS under Section 7 of the MSA is to protect the interests of beneficiaries at all times. Clinically incorrect non-adherence letters — generated by an unremediated, documented integration gap — directly and materially prejudice beneficiaries. The elderly GEMS member in question received communications falsely characterising her as non-adherent despite consistent chronic medication claims. This is a concrete, documented violation of beneficiary protection that Section 7 mandates the CMS to address.

2.2 Section 1 and Section 47 — Maladministration

"Maladministration" under the MSA encompasses any act or omission by an administrator that causes prejudice to a member. The RCA confirms that the integration gap is architectural in nature: it is not a one-time event but a recurring structural risk that generates fresh acts of maladministration with every automated non-adherence cycle. Each day the system remains uncorrected constitutes a new and independent act of maladministration under Section 47. The CMS is mandated to investigate and direct remediation.

2.3 Section 58 — Accreditation Standards and IT System Integrity

Medscheme operates as an Accredited Administrator by virtue of CMS accreditation. Version 7 of the CMS Accreditation Standards for Administrators requires, inter alia, that administration systems maintain IT integrity and be fit for purpose in managing the complexity of the schemes under administration. A Priority 1 integration gap — formally documented, internally escalated, and unremediated across multiple claims cycles — constitutes a material breach of these accreditation standards. The CMS is not merely empowered to review Medscheme's accreditation compliance in light of this finding; it is obligated to do so.

2.4 Section 44 — Power of Forensic Inspection

The CMS is now on formal notice that my Section 162 Director Delinquency Application (HCGS951842) pleads, as a primary ground, that the AfroCentric / Medscheme Board received my Priority 1 Material Risk Report — documenting the Gexus-DMS failure — and suppressed rather than remediated it. Board-level suppression of a material patient safety risk is the precise trigger for which Section 44 inspection powers exist. The Registrar has the authority to order a forensic inspection of the Administrator's affairs. Given the confluence of a documented system failure, a formal High Court delinquency application, and the Board's confirmed knowledge of the risk, this office is respectfully requested to consider whether a Section 44 inspection is warranted.

3. THE DISCOVERY HEALTH PRECEDENT — CMS JURISDICTION OVER TECHNICAL SYSTEM FAILURES IS ESTABLISHED

The CMS has already exercised its supervisory jurisdiction over a technical system failure at a medical scheme administrator in 2026. That precedent forecloses any argument that this Council lacks jurisdiction or authority to engage with the Gexus-DMS integration failure now before it.

In early January 2026, it was reported that the Discovery Health Medical Scheme (DHMS) had experienced a pharmacy claims processing error affecting approximately 16,507 members across its Executive, Comprehensive, and Priority plans. Certain claims were incorrectly processed at 100% of the Discovery Health Rate from the Above Threshold Benefit (ATB) rather than from the Medical Savings Account or Self-Payment Gap, resulting in an estimated R130–170 million in overpaid benefits across most of 2025.

The CMS confirmed on 5 January 2026 that it had become aware of the matter through media reports and that it would address the concerns within its statutory mandate. Following a formal escalation by consumer advocate MediCheck — raising compliance concerns under Regulation 6, Section 57, and Regulation 17 of the MSA — the CMS placed the recovery process under active scrutiny and confirmed governance expectations around transparency, fairness, and member communication.

Sources: *Moonstone Information Refinery*, 8 January 2026: "CMS steps in as Discovery Health begins recovering pharmacy overpayments" (moonstone.co.za); *MedicalBrief*, 14 January 2026: "Discovery faces CMS probe as it backtracks on overpayments" (medicalbrief.co.za).

The regulatory significance of this precedent for the present matter is as follows:

- The DHMS error was a financial processing glitch — claims paid from the wrong benefit pocket. The Gexus-DMS failure is a clinical data integrity error — non-adherence status incorrectly flagged, generating clinically incorrect letters to vulnerable beneficiaries. If the CMS exercised jurisdiction over the former, it cannot in good conscience decline jurisdiction over the latter, which carries direct clinical implications for patient safety.
- The CMS intervened in the DHMS matter without waiting for member complaints to be formally registered — it acted on media reports and a third-party advocacy escalation. In the present matter, the CMS has before it a formal disclosure, a Root Cause Analysis, an internal Medscheme investigation record confirming the integration gap, and a High Court filing. The threshold for engagement is materially higher here, not lower.
- MediCheck's escalation to CMS cited Section 57 (governance and administration oversight) and Regulation 17. These are the same supervisory instruments that authorise the CMS to scrutinise Medscheme's system integrity and accreditation compliance in the present matter.

The CMS cannot credibly maintain that technical system failures at administrators fall outside its jurisdiction when it has demonstrably exercised that jurisdiction within the past three months. The Discovery Health precedent is not an analogy — it is a direct and binding illustration of this Council's own practice.

4. ANTICIPATED JURISDICTIONAL OBJECTIONS — PRE-EMPTIVELY ADDRESSED

4.1 "This is an isolated incident"

The RCA documentation refutes this characterisation definitively. The integration gap is hardcoded into the system architecture: every member who transitions from acute to chronic benefit enters the same tracking blind spot in DMS. The risk is predictable, recurring, and structural. It is not an accident — it is a design flaw that generates fresh maladministration with each automated cycle. Unlike the DHMS matter — which was a processing error corrected in a single reprocessing run — the Gexus-DMS failure has remained unaddressed across multiple automated non-adherence cycles.

4.2 "The matter is sub judice"

The High Court application (HCGS951842) concerns the delinquency of directors who suppressed the risk report. The CMS's regulatory mandate to protect beneficiaries from an active, uncorrected system failure is independent of, and not precluded by, civil proceedings against individuals. The CMS did not decline to act in the DHMS matter on the basis that Discovery Health had received member complaints or legal challenges — it acted on the live risk. The same logic applies here.

4.3 "The CMS previously declined jurisdiction"

The February 2026 declination addressed a different characterisation of the complaint. The present disclosure is squarely framed as a challenge to Administrator infrastructure and accreditation compliance — matters over which the CMS has primary and exclusive jurisdiction. The previous

referral to HPCSA/SANC does not constitute a final determination on the Administrator Accreditation and maladministration dimensions now formally raised.

5. SUMMARY OF STATUTORY DUTIES NOW ENGAGED

Disclosure Element	CMS Statutory Duty	MSA Provision
Systemic Integration Error (Gexus-DMS)	Investigate Maladministration & System Integrity	Sections 1, 47
Ongoing Patient Safety Risk to 1.9m GEMS Members	Protect Beneficiary Interests	Section 7
Priority 1 Risk Suppression at Board Level	Review Fitness for Accreditation	Section 58
High Court Director Delinquency Application (HCGS951842)	Consider Forensic Inspection	Section 44

6. FORMAL REQUESTS

In light of the foregoing, I respectfully request that the CMS:

- Formally re-open the investigation of this disclosure under the Maladministration and Administrator Accreditation dimensions of the MSA, distinct from the professional conduct referral previously issued;
- Direct Medscheme Holdings (Pty) Ltd to provide a written account of the current status of the Gexus-DMS integration gap, including the date on which it was identified, the remediation steps taken (if any), and the timeline for full remediation;
- Assess whether Medscheme's continued operation of an unremediated, documented Priority 1 system failure is consistent with its obligations under the CMS Accreditation Standards Version 7, specifically the IT Integrity and Clinical Governance requirements;
- Consider whether the Board-level suppression of the Priority 1 Material Risk Report — as pleaded in High Court Case HCGS951842 — constitutes sufficient grounds for the Registrar to exercise inspection powers under Section 44 of the MSA; and
- Provide a substantive written response to this office within fifteen (15) business days, acknowledging receipt of this formal re-engagement and confirming the Council's intended course of action.

7. CONSEQUENCE OF NON-RESPONSE

Should the CMS decline to engage substantively with this disclosure, or issue a further declination without addressing the Administrator Accreditation and maladministration dimensions raised herein, I intend to place the following on formal record in the High Court proceedings (HCGS951842) and in my ongoing SAHRC file:

- That the CMS, having been placed on formal notice of a documented Priority 1 integration failure generating clinically incorrect communications to GEMS beneficiaries, elected not to exercise its statutory mandate under Sections 7, 47, and 58 of the MSA;
- That the Council's inaction, in the context of a confirmed, unremediated systemic risk to 1.9 million GEMS members, is itself a dereliction of the Council's statutory duty; and
- That this dereliction will be brought to the attention of the Portfolio Committee on Health, the Parliamentary Oversight Mechanism, and such other supervisory bodies as may be appropriate.

It is not my intention to escalate unnecessarily. It is my intention to ensure that a known, documented, patient safety risk does not continue to be deflected between regulatory bodies while remaining unaddressed.

I remain available to provide the full RCA documentation, the High Court application bundle (HCGS951842), and any further technical or governance information this office requires to discharge its statutory mandate.

Yours faithfully,

WILBUR WILLIAM MATTHEE

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